

DATE OF CONSULTATION: February 28, 2023
PATIENT: RIVERA, Frances (*28.12.1959)
MRN: AML064

CONSULTING SERVICE: Hematology-Oncology (Dr. B. Carter)

REASON FOR CONSULTATION: Goals of Care Discussion and Management of Refractory Pain in a 63-year-old female with end-stage Acute Myeloid Leukemia.

BACKGROUND:

I had the privilege of meeting Ms. Rivera and her two adult daughters today. Ms. Rivera is a 63-year-old woman with a diagnosis of adverse-risk AML, characterized by **monosomy 7**, diagnosed on May 27, 2022. Given her age and the high-risk nature of her disease, she was initiated on a non-intensive regimen of **Venetoclax and Azacitidine** starting on June 3, 2022.

She achieved an initial complete remission with incomplete count recovery (CRi) and was able to maintain a reasonable quality of life for approximately six months. However, in January 2023, she developed progressive pancytopenia, fatigue, and significant bone pain. A repeat bone marrow biopsy confirmed a frank relapse of her leukemia, which was now refractory to the Venetoclax/Azacitidine regimen.

The oncology team discussed salvage therapy options including allogeneic stem cell transplantation, but given the extremely poor prognosis associated with relapsed monosomy 7 AML and her declining performance status (ECOG 3), Ms. Rivera made the clear and courageous decision to decline further anti-leukemic therapy and transition her goals of care to focus entirely on comfort, quality of life, and time with her family.

ASSESSMENT OF SYMPTOM BURDEN:

- **Pain:** This is her most distressing symptom. She describes a constant, deep, aching pain in her lower back and hips, consistent with leukemic infiltration of the bone marrow. She rates it a 9/10 at its worst and 6/10 at its best. It is significantly impacting her ability to rest, reposition in bed, and interact with family. Her current regimen of Tramadol 50 mg every 6 hours is providing minimal relief.
- **Dyspnea:** She experiences shortness of breath with minimal exertion, such as speaking a full sentence. This is multifactorial, related to severe anemia (Hgb 6.8 g/dL) and general deconditioning.
- **Fatigue:** Profound and debilitating. She spends >75% of her day in bed or a chair.
- **Anxiety/Existential Distress:** Ms. Rivera expresses an appropriate level of sadness about her prognosis but also significant anxiety about the dying process, particularly the fear of being in pain.

GOALS OF CARE DISCUSSION:

I spent considerable time with Ms. Rivera and her daughters. She verbalizes a clear understanding of her terminal diagnosis and that her remaining time is likely short (weeks). Her primary goals are:

1. To be "as comfortable as possible."
2. To remain lucid enough to interact with her grandchildren.
3. To die at home, if feasible.

She has a signed DNR/DNI order in her chart, which she reaffirmed. She does not want any further life-prolonging interventions, including antibiotics for new infections. She consents to receive blood product transfusions if they will directly alleviate symptoms (e.g., pRBCs for dyspnea), but not simply to treat a number on a lab report.

DATE OF CONSULTATION: February 28, 2023
PATIENT: RIVERA, Frances (*28.12.1959)
MRN: AML064

RECOMMENDATIONS:

1. Pain Management (Aggressive Titration):

- **Discontinue Tramadol.**
- **Initiate a full opioid agonist.** Start scheduled long-acting Morphine (MS Contin) 15 mg PO every 12 hours.
- **For breakthrough pain:** Start immediate-release Morphine solution 5-10 mg PO every 3 hours as needed. Titrate aggressively for comfort. We will follow up in 24 hours to assess efficacy and adjust dosing. The goal is a pain score of < 4/10.

2. Dyspnea Management:

- Transfuse 1 unit of pRBCs now for symptomatic anemia.
- Initiate low-dose Morphine (2.5 mg PO every 4 hours), which is excellent for the sensation of air hunger.
- Provide supplemental oxygen via nasal cannula at 2 L/min for comfort.

3. Anxiety Management:

- Start scheduled Lorazepam 0.5 mg PO every 8 hours. Provide an additional 0.5 mg every 4 hours as needed for episodes of panic or severe anxiety.

4. Psychosocial & Spiritual Support:

- Engaged our team's social worker to begin coordinating with a home hospice agency immediately.
- Offered a visit from our hospital chaplain, which the patient accepted.

5. Medication Reconciliation: Discontinue all non-essential medications (e.g., statin, antihypertensives if BP allows) to reduce pill burden.

I have communicated this comprehensive plan to the primary oncology team and the nursing staff. Our team will follow Ms. Rivera daily while she is in the hospital to ensure her symptoms are controlled and to facilitate a smooth and timely transition to home hospice care.

(Follow-up Note: Ms. Rivera was successfully discharged to home hospice 3 days later. The hospice agency reported that she passed away peacefully at home 1 week after discharge, with her symptoms well-managed.)